Ribociclib + endocrine therapy · metastatic

> **Confirm the individual supportive prescription.** This review aid does not generate a medication order. Use the current NCCP regimen, antiemetic guidance and local pharmacy plan; assess interactions, allergies and comorbidities.

Antiemetic and premedication review

- Oral moderate–high category; review QT interactions before antiemetic selection
- The ribociclib break is Days 22–28. Your endocrine partner usually continues.
- Early blood tests and a Day-14 ECG are important even if you feel well.

Additional supportive care

- The ribociclib break is Days 22–28. Your endocrine partner usually continues.
- Early blood tests and a Day-14 ECG are important even if you feel well.
- Avoid grapefruit; check new prescriptions, over-the-counter medicines and supplements for interactions.
- Do not take an extra dose. Take the next prescribed dose at the usual time and contact the team if uncertain.

Monitoring and handover

- ECG: an electrical heart tracing: Before starting and around Day 14 of cycle 1; then as indicated. Starting QTcF must be <450 ms. This is ECG monitoring, not a routine 3-monthly echo programme. Review QT-prolonging/CYP3A medicines and electrolyte abnormalities.
- Bloods and electrolytes: Bloods every 2 weeks for cycles 1–2; before cycles 3–6; then as indicated. Electrolytes before each of the first six cycles, then as indicated; intensify monitoring with abnormal results.
- Cancer-response imaging: Usually every 2–4 months, adapted to the clinical picture.
- Markers, if selected: Alongside the individual response plan.
- Bone health with endocrine therapy: At AI/OFS initiation and then by risk. Repeat DEXA is based on baseline bone density, fracture risk and bone treatment. A 12–24-month reassessment can be appropriate near a treatment threshold or when risk changes; it is not an automatic annual scan for everyone.
- Week off ribociclib: Days 22–28 of each cycle. This metastatic schedule starts at 600 mg/day unless adjusted. Do not substitute the separate 400 mg/3-year adjuvant ribociclib pathway.

Access, visits and patient preparation

- Interactions and storage: Check all new medicines and supplements with oncology/pharmacy, avoid grapefruit, store as labelled and keep away from children. Return unwanted tablets to the pharmacy.
- Endocrine and ovarian-suppression visits: An aromatase inhibitor is generally a daily tablet. Fulvestrant, when selected for metastatic treatment, has loading injections on Days 1, 15 and 29 then monthly. Ovarian-suppression injections have their own prescription and visit schedule.
- No infusion pump: The CDK4/6 medicine is taken orally. An implanted port or infusion pump is not required for these tablets; other treatment may have separate access needs.

Patient handover

Explain the current phase, how to take each medicine, which tests are expected next and who arranges and reviews them. Confirm urgent contact details. Provide the patient portal, visual guide, passport, treatment card and whole-treatment map.

Fever, shivers or suddenly feeling unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: emergency help, 112 / 999.

Sources

NCCP 00525 v6 · official regimen

NICE NG101 · bone health and follow-up

NOGG · section 7, aromatase inhibitors and fracture risk

Novartis · Kisqali product information, updated September 2026

NCCP supportive-care antiemetics for NCIS

SACTCheck v0.83.7 · Sources checked 2026-10-17 · Draft — independent clinical, pharmacy and patient/PPI review pending